

Juvenile Arthritis: A Parent's Guide to Childhood Joint C

Comprehensive UK guide to juvenile idiopathic arthritis (JIA). Symptoms in children, diagnosis pathway, treatment options, school support and NHS resources for parents.

By **Living With Arthritis Editorial Team** · Reviewed by **Clinical Advisory Panel** (HCPC-registered physiotherapy & rheumatology nursing input) · 2026-01-24 · livingwitharthritis.org.uk

Arthritis doesn't just affect adults. In the UK, approximately **15,000 children and young people** live with juvenile idiopathic arthritis (JIA) — the most common type of childhood arthritis. Discovering that your child has arthritis can be frightening and overwhelming. This guide provides UK parents with comprehensive information about JIA: what it is, how it's diagnosed, treatment options, and how to support your child at school and at home.

What Is Juvenile Idiopathic Arthritis?

Juvenile idiopathic arthritis (JIA) is a group of autoimmune conditions that cause joint inflammation in children under 16. "Idiopathic" means the cause is unknown — though genetics and environmental triggers are thought to play a role. JIA is **not** the same as adult rheumatoid arthritis, though some forms share similarities. It's also not caused by injury, diet or anything the child or parent did.

Types of JIA

- Oligoarticular JIA — Affects 4 or fewer joints. Most common type (about 50% of cases). Often affects knees or ankles. Can cause eye inflammation (uveitis) — regular eye screening is essential
- Polyarticular JIA — Affects 5 or more joints. Can be rheumatoid factor positive or negative. May affect hands, feet and larger joints
- Systemic JIA (Still's disease) — Affects the whole body, causing spiking fevers, rash, joint inflammation and sometimes organ inflammation. The most severe form

- Enthesitis-related JIA — Involves inflammation where tendons attach to bone (enthesitis). More common in boys. May involve the spine and sacroiliac joints
 - Psoriatic JIA — Associated with psoriasis skin disease. May include dactylitis (swollen "sausage" fingers or toes)
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Recognising Symptoms



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Children with JIA may not always complain of pain — instead, parents often notice behavioural or functional changes:

- Limping — Especially in the morning or after rest, without any injury
- Morning stiffness — Difficulty moving in the morning that improves through the day

- Swollen joints — Warm, puffy joints, often knees, ankles or wrists
- Reluctance to use a limb — Avoiding using one hand, or crawling instead of walking in toddlers
- Irritability — Young children may become grumpy or clingy rather than describing pain
- Fatigue — Unusual tiredness or reduced activity levels
- Fever — In systemic JIA, a daily spiking fever (often in the evening)
- Rash — A salmon-pink rash that comes and goes (systemic JIA)
- Eye problems — Usually symptomless initially — detected through routine screening

When to See Your GP

See your GP if your child has joint swelling, limping or stiffness lasting more than 2 weeks without obvious cause. Early referral to a paediatric rheumatologist is crucial — NICE guidelines recommend children with suspected JIA should be seen by a specialist within **6 weeks** of GP referral.

Diagnosis

There is no single test for JIA. Diagnosis is based on clinical assessment by a paediatric rheumatologist, including a thorough joint examination, blood tests (CRP, ESR, rheumatoid factor, anti-CCP, ANA, full blood count), imaging (ultrasound, MRI — X-rays may be normal early on), and exclusion of other conditions (infections, injuries, other autoimmune conditions). Blood test results may be normal in some types of JIA — the diagnosis is primarily clinical.

Treatment



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Modern treatment aims to achieve **remission** — complete control of inflammation to prevent joint damage and allow normal growth and development.

First-Line Treatment

- NSAIDs — Ibuprofen or naproxen for pain and inflammation
- Steroid injections — Directly into affected joints for rapid, targeted relief. Often the first treatment for oligoarticular JIA. May be done under sedation for young children

Disease-Modifying Drugs

- Methotrexate — The most commonly used DMARD for JIA. Usually given as a weekly injection (liquid for young children). Folic acid is prescribed alongside. Side effects may include nausea, mouth ulcers and fatigue
- Sulfasalazine — Sometimes used as an alternative to methotrexate

Biologic Therapies

- Anti-TNF drugs — Etanercept, adalimumab. Used when methotrexate alone is insufficient
- Tocilizumab — Particularly effective for systemic JIA
- Abatacept — For polyarticular JIA not responding to other treatments

These medications can seem daunting, but they are well-established, carefully monitored, and have transformed outcomes for children with JIA. Without treatment, uncontrolled inflammation can damage joints, affect growth and cause permanent disability.

Physiotherapy

A crucial part of JIA management. Paediatric physiotherapists provide exercises to maintain joint range of motion, strengthening programmes to support growing joints, hydrotherapy (pool therapy), splints for affected joints (e.g., wrist splints during flares), and advice on sports and physical activity. Children with JIA should be encouraged to be as **physically active** as possible — most sports are safe and beneficial.

School Support

JIA is covered by the Equality Act 2010, meaning schools must make reasonable adjustments. A school health plan should be arranged, and adjustments may include extra time between lessons (to avoid rushing between classrooms), access to a lift if stairs are painful, a second set of textbooks at home (to avoid carrying heavy bags), modified PE activities (inclusion, not exclusion — adapted activities rather than sitting out), permission to stand or move during lessons if stiffness is an issue, exam access arrangements (extra time, rest breaks, use of a laptop for writing), and a quiet space to rest during flare-ups.

Communicating with School

Meet with your child's teacher and SENCO (Special Educational Needs Coordinator) at the start of each year. Provide written information about JIA. Explain that the condition is invisible and variable — your child may look well but be in significant pain. Ask the school to liaise with the paediatric rheumatology team if needed.

Emotional Support



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JIA has a significant emotional impact on children and families. Children may feel different from peers, frustrated by limitations, anxious about medical procedures, worried about the future, or struggling with visible side effects of medication. Psychological support should be available through the paediatric rheumatology team. Charities like CCAA (Children's Chronic Arthritis Association) and JIA@NRAS provide peer support, family events and age-appropriate information.

Prognosis

The outlook for JIA has improved dramatically with modern treatment. With early, aggressive treatment, many children achieve remission and have no long-term joint damage.

Approximately 50% of children with oligoarticular JIA go into permanent remission. Polyarticular and systemic JIA may require longer-term treatment but can be well controlled. Regular monitoring throughout childhood and transition to adult rheumatology services at age 16–18 ensures continuity of care.

UK Resources for Parents

- Arthritis UK — JIA resources, family weekends and youth advisory panel
- JIA@NRAS — Part of the National Rheumatoid Arthritis Society, specifically for JIA families
- CCAA — Children's Chronic Arthritis Association, offering support days and information
- Young Arthritis — Arthritis UK's youth programme for teens and young adults
- Family Fund — Grants for families with disabled children for equipment, holidays and respite
- Contact — Charity for families with disabled children, offering advice on benefits, education and services

JIA is challenging for the whole family, but with modern treatment and support, most children with JIA can live full, active lives. Our virtual assistant can answer questions about childhood arthritis and signpost UK support services.

This article is for general information only. If you're concerned about your child's joints, see your GP for assessment and referral to a paediatric rheumatologist.

Q&A

Question: What Is Juvenile Idiopathic Arthritis?

Short answer: Juvenile idiopathic arthritis (JIA) is a group of autoimmune conditions that cause joint inflammation in children under 16. "Idiopathic" means the cause is unknown — though genetics and environmental triggers are thought to play a role. JIA is not the same as adult rheumatoid arthritis, though some forms share similarities. It's also not caused by injury, diet or anything the child or parent did.

Question: When should I seek professional advice?

Short answer: If symptoms persist or worsen, pain is severe and unresponsive to usual relief, or you develop new symptoms, contact your GP, physiotherapist or rheumatology team promptly.

Question: Where can I get free arthritis support in the UK?

Short answer: Arthritis UK: 0800 5200 520. NRAS: 0800 298 7650. Free tools and guides at livingwitharthritis.org.uk.

General information only — not medical advice. Consult your GP or specialist before changing medication or starting a new programme. Emergency: 999. NHS advice: 111. Living With Arthritis · livingwitharthritis.org.uk